Ch 97 · Peripartum cardiomyopathy

Definition

- LVEF < 45% late pregnancy to months postpartum, no other cause
- Risks: African ancestry, pre-eclampsia, multiple pregnancy, age, multiparity; 16-kDa prolactin

Treatment

- Postpartum GDMT (enalapril safe in breastfeeding); no ACEi/ARNI in pregnancy
- Bromocriptine + anticoagulation (BOARD); anticoagulate if EF < 30–35%, thrombus or AF
- Shock: levosimendan, early MCS; avoid high-dose catecholamines; wearable defibrillator

Prognosis

- ~50% recover by 6 months; future pregnancy risky if EF not normal

Key point: Breathlessness postpartum needs an echo, not just reassurance.

Ch 98 · Amniotic fluid embolism

Presentation

- Labour/within 30 min postpartum: sudden hypoxia, hypotension/arrest, DIC
- Biphasic: RV failure from pulmonary hypertension, then LV failure

Resuscitation

- ALS with uterine displacement; resuscitative hysterotomy within 5 min if ≥ 20 weeks
- RV support (noradrenaline, inotropes, inhaled vasodilators), avoid overload
- Aggressive fibrinogen, FFP, platelets, TXA; viscoelastic testing; VA-ECMO

Diagnosis

- Clinical exclusion: PE, anaphylaxis, haemorrhage, eclampsia, high spinal, LA toxicity

Key point: Expect coagulopathy early and prepare products at once.

Ch 99 · Respiratory failure in pregnancy

Physiology

- ↑ minute ventilation; normal $PaCO_2$ ≈ 4.0 kPa; ↓ FRC, ↑ O_2 consumption → fast desaturation
- Airway oedema, aspiration risk

Airway

- Experienced operator, ramp, uterine displacement, HFNO/NIV pre-oxygenation
- RSI, smaller tube, videolaryngoscope, obstetric difficult airway plan

Ventilation targets

- SpO_2 ≥ 95% (PaO_2 > 9.3–10 kPa); $PaCO_2$ 4.0–4.7 kPa
- Lung-protective ventilation; prone and ECMO feasible; delivery may help

Key point: A 'normal' $PaCO_2$ of 5.3 kPa is hypercapnia in late pregnancy.

Ch 100 · Pre-eclampsia, eclampsia and HELLP

Definitions

- Hypertension after 20 weeks + proteinuria or organ dysfunction
- Severe: BP ≥ 160/110, low platelets, liver/renal failure, pulmonary oedema, neuro signs
- HELLP = haemolysis, elevated liver enzymes, low platelets

Management

- Magnesium 4 g load + 1 g/h × 24 h (MAGPIE); extra 2 g for recurrence; watch reflexes/RR
- BP < 150/100: IV labetalol, hydralazine, oral nifedipine
- Delivery after stabilisation; fluid restriction ~80 mL/h

Complications

- ICH, PRES, pulmonary oedema, AKI, liver haematoma/rupture, DIC, abruption

Key point: Magnesium prevents seizures; antihypertensives prevent strokes.

Ch 101 · Obstetric haemorrhage

4 Ts

- Tone (70%), trauma, tissue, thrombin

Management

- Massage, oxytocin, ergometrine, carboprost, misoprostol
- TXA 1 g within 3 h (WOMAN); early fibrinogen (keep > 2 g/L), FIBTEM-guided
- Balloon tamponade, B-Lynch, embolisation, hysterectomy

Fibrinogen

- < 2 g/L predicts severe PPH; falls earlier than other factors

Key point: Call for help early and run treatments in parallel.

Ch 102 · Sepsis in pregnancy

Recognition

- Physiological tachycardia, low BP, raised WBC mask sepsis; use MEOWS/omqSOFA; lactate valid

Sources

- Endometritis, chorioamnionitis, septic abortion, wound, mastitis, pyelonephritis, pneumonia
- Group A strep can kill within hours

Management

- Hour-1 bundle; pip-taz + clindamycin for GAS
- Source control (retained products, debridement, hysterectomy); IVIG for toxic shock
- Fetal monitoring and timing of delivery with obstetrics

Key point: Diarrhoea and severe pain postpartum may be group A streptococcus.